NCCP 00321 · VERSION 9

Your treatment, explained

Gastrointestinal cancers · NCCP 00321 v9 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

CAPOX / XELOX

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, breathing difficulty, severe diarrhoea, inability to drink, reduced urine, uncontrolled bleeding or a rapidly worsening symptom needs prompt assessment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · hse

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment plan
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Treatment aim and duration
- Taking capecitabine safely
- What the checks are for
- Separate the symptoms from the cause
- Living with treatment
- The names on your treatment plan
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

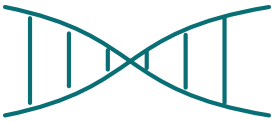
Your medicines and what they do

CAPOX, also called XELOX, combines capecitabine tablets with oxaliplatin infusion. This NCCP protocol covers adjuvant stage III colon cancer, advanced colorectal cancer and adjuvant gastric cancer after D2 surgery. Confirm the treatment aim and duration for your indication.

ORAL CHEMOTHERAPY

Capecitabine

Xeloda



Becomes fluorouracil in the body

The body converts capecitabine into fluorouracil, which disrupts DNA and RNA processes needed for cell growth. Tablets are active chemotherapy; taking them at home still requires a clear monitoring and interruption plan.

Swallowed tablet → Converted to fluorouracil → Cell growth disrupted

What to watch for

Stop tablets and contact the team for significant diarrhoea, painful mouth ulcers, hand-foot soreness affecting activities, fever or dehydration. Chest pain needs urgent assessment. Do not restart until instructed.

Sources: nccp · patient

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links DNA strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

Sources: nccp · patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your treatment plan

INFUSION + ORAL CHEMOTHERAPY

21-day treatment cycle

D1 D14 D15 D21

Oxaliplatin on Day 1; capecitabine twice daily on Days 1–14, followed by Days 15–21 without capecitabine. Follow your individual prescription.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

Check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Tablets / hands and feet	Confirm written dose, tablet supply and hold instructions	Review adherence, mouth, bowel and skin symptoms	Days 1–14 tablets; Days 15–21 planned break unless team changes it	Do not restart held tablets or replace missed doses without advice	Return unused tablets as instructed; follow persistent symptoms
Neuropathy / cold sensitivity	Record existing nerve symptoms	Check persistence and effect on daily tasks	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks	Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
INR if taking warfarin	Check anticoagulant and baseline INR	If taking warfarin: baseline and regular INR monitoring as clinically indicated.	Report bleeding or bruising	Adjust warfarin only as prescribed	Continue the agreed anticoagulation plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00321 v9 · official regimen

THE WHOLE TREATMENT JOURNEY

CAPOX / XELOX: treatment and checks

Gastrointestinal cancers

Oxaliplatin on Day 1; capecitabine twice daily on Days 1–14, followed by Days 15–21 without capecitabine. Follow your individual prescription.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
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What each check is for

1

Confirm your personal plan

Before starting

CAPOX, also called XELOX, combines capecitabine tablets with oxaliplatin infusion. This NCCP protocol covers adjuvant stage III colon cancer, advanced colorectal cancer and adjuvant gastric cancer after D2 surgery. Confirm the treatment aim and duration for your indication.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00321

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

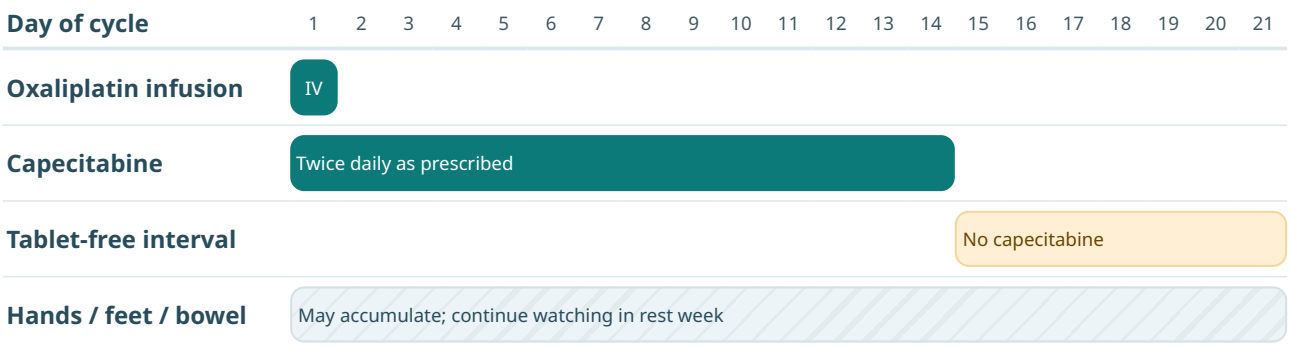
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Oxaliplatin on Day 1; capecitabine twice daily on Days 1–14, followed by Days 15–21 without capecitabine. Follow your individual prescription.



Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Blood counts / infection	Act on symptoms on any day																				

Treatment and rest: The prescribed capecitabine course is followed by a tablet-free interval. Follow your own written start and stop instructions.

Blood counts: Infection can occur on any day, including the tablet-free week. Fever or sudden illness needs prompt contact.

Hands, feet and bowel: Symptoms may accumulate during the tablet course and can continue into the rest week. Follow your stop-and-call instructions.

Cold and nerve symptoms: Oxaliplatin can cause cold-triggered symptoms, and numbness may accumulate over cycles. Report functional changes before the next treatment.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00321 v9 · official regimen ↗ · eviQ: patient information (supportive explanation; NCCP schedule takes priority) ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Read the whole cycle as text

Day 1: Infusion and tablet plan

Your team checks symptoms, blood results and the oral prescription. Make sure you know the tablet strengths, number per dose and exact start and stop instructions. Use the written prescription. Take capecitabine with water within 30 minutes after food; never guess the tablet count.

Day 2–7: Tablets at home

Continue twice-daily capecitabine according to the prescription. A missed or vomited dose is not replaced. Record doses missed and contact the team if you are unsure. Do not double the next dose. Keep tablets in the original packaging, away from children; do not crush or split unless the oncology pharmacist has arranged it.

Day 8–14: Watch for accumulating symptoms

Diarrhoea, mouth ulcers or painful hands and feet can develop or worsen. Poor intake and dehydration can worsen kidney function and increase risk from capecitabine. Stop tablets and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, fever, dehydration or chest symptoms. Do not restart without advice.

Day 15–21: The prescribed tablet-free week

No capecitabine is planned in this interval, but side effects and infection can still occur. Keep recording symptoms and confirm blood-test and review arrangements. Do not use leftover tablets to make up missed doses or start the next cycle early. The next cycle requires team confirmation.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.



STEP 2

Taking the tablets

Use the prescribed morning and evening dose within 30 minutes after food on the agreed treatment days. Do not make up a missed or vomited dose. Follow the team's hold instructions for significant diarrhoea, mouth soreness or painful hands and feet, and contact them promptly.



STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.



STEP 4

Getting help between visits

Follow your oncology unit's urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00321 v9 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Nausea prevention and rescue

Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

2 Bowel symptoms

Report diarrhoea promptly and follow the written rescue plan. Severe symptoms, fever, dehydration or inability to drink need urgent advice.

3 Counts and recovery

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

4 Cold and nerve symptoms

Avoid cold exposure as advised after oxaliplatin. Report persistent numbness or difficulty with buttons, walking or everyday tasks before the next dose.

5 **Skin and mouth care**

Use gentle skin and mouth care. Painful hands and feet or mouth sores can require a tablet hold and team review; do not continue simply to finish the pack.

6 **Warfarin monitoring**

If taking warfarin: baseline and regular INR monitoring as clinically indicated.

Sources: nccp

TREATMENT REFERENCE · 01

Treatment aim and duration

CAPOX is used in several distinct settings.

Adjuvant colon cancer

Following resection of stage III colon cancer, CAPOX aims to reduce recurrence risk. Duration is a discussion about cancer risk and toxicity; three and six months are not interchangeable for everyone.

Sources: nccp

Advanced colorectal cancer

The aim may be cancer control, symptom relief and longer survival. Molecular findings, previous therapy and fitness affect the overall plan. Additional targeted medicines require their own regimen and counselling.

Sources: nccp

After gastric surgery

The adjuvant gastric evidence concerns a specific postoperative setting after D2 gastrectomy. Nutrition, postoperative recovery and oral tolerance need particular attention.

Sources: nccp

Alternatives and review points

Ask why this oral/infusion combination was chosen, whether an infusional alternative is suitable, and what would prompt a pause or change. Taking tablets at home does not mean the treatment is mild.

Sources: nccp

TREATMENT REFERENCE · 02

Taking capecitabine safely

Your prescription is the authority for dose and dates.

Food, doses and tablet handling

Take doses about 12 hours apart and within 30 minutes after food, as instructed. Swallow whole with water. If swallowing is difficult, ask the oncology pharmacist for a safe plan. Do not crush tablets or improvise a liquid.

Sources: patient

Missed or vomited doses

Do not replace a missed or vomited dose and do not take an extra dose. Record it and resume the next scheduled dose only if otherwise safe under your written instructions. Repeated vomiting or uncertainty needs a call.

Sources: patient

When to stop and call

Do not continue through significant diarrhoea, painful mouth sores, painful hand-foot symptoms, fever, dehydration or chest pain. Stop capecitabine and obtain prompt advice. A clinician decides whether and how to restart.

Sources: patient

The rest week is part of treatment

Days without tablets are planned. Do not fill them with leftovers. Return unused medicine through the hospital/pharmacy process and keep it out of household waste and away from other people.

Sources: patient

TREATMENT REFERENCE · 03

What the checks are for

Symptoms, blood results and the treatment aim answer different questions.

Blood counts

Neutrophils protect against infection, haemoglobin carries oxygen and platelets help clotting. A normal previous result or feeling well does not prove the current counts are safe. Fever or sudden illness needs immediate advice.

Sources: hse

Kidney and liver function

Organ function can change drug exposure. Vomiting or diarrhoea can cause dehydration and worsen kidney function. Report reduced urine, poor intake, jaundice and new medicines. The clinical assessment combines results with symptoms.

Sources: nccp

Describe symptoms in practical terms

Tell the team about the worst symptoms since the last visit, even if improved today. Examples include extra stools above your usual pattern, whether mouth pain affects drinking, and whether numbness affects buttons or balance.

Sources: patient

How benefit is assessed

After surgery, treatment aims to reduce the chance of recurrence; there may be no visible cancer to measure. In advanced disease, scans, symptoms and function help assess control. Ask when review is planned and what would lead to a change.

Sources: nccp

Separate the symptoms from the cause

You do not need to diagnose an adverse effect before calling.

Hand-foot syndrome versus neuropathy

Capecitabine can cause tender, red or peeling palms and soles. Oxaliplatin more often causes cold-triggered tingling and cumulative numbness. Both can affect walking or hand use. Tell the team about both the sensation and what you can no longer do.

Sources: patient

Diarrhoea and dehydration

Record stools above your own usual pattern, night-time symptoms and stoma output if relevant. Reduced urine, dizziness, inability to drink or worsening diarrhoea needs prompt advice. A medicine to stop diarrhoea does not replace the interruption plan.

Sources: patient

Mouth, appetite and fatigue

Mouth soreness can make food and fluid difficult. Start gentle mouth care and report painful swallowing or reduced intake. Ask for symptom and nutritional support. New major weakness or breathlessness needs assessment.

Sources: hse

Heart or infusion symptoms

Fluoropyrimidines can cause serious cardiac effects; oxaliplatin can cause allergic reactions. New chest symptoms, fainting or breathing difficulty need urgent assessment. Do not assume these are anxiety or ordinary cold sensitivity.

Sources: patient

TREATMENT REFERENCE · 05

Living with treatment

Plan around the activities and people that matter to you.

Food, drink and practical help

Use foods and drinks you can manage. Tell the team about weight loss, poor intake or difficulty shopping and cooking; ask for dietitian or practical support. A fixed daily fluid target is not appropriate for everyone, particularly with heart or kidney problems.

Sources: patient

Activity, rest and driving

Balance rest with manageable activity. Record what fatigue stops you doing. Do not drive when dizzy, unwell or affected by sedating medicines. Ask about work adjustments and transport for treatment days.

Sources: patient

Medicines, vaccines and supplements

Bring a complete medicine list, including over-the-counter products and supplements. Ask the oncology pharmacist before changes. Discuss vaccination timing. Do not stop prescribed long-term medicines on your own.

Sources: hse

Fertility, contraception and breastfeeding

Discuss fertility preservation before treatment when relevant. Ask for a written contraception and breastfeeding plan covering each medicine and the period after treatment. Tell the team promptly about a possible pregnancy.

Sources: hse

Support for you and your family

Ask about psycho-oncology, social work, counselling and symptom support. Take information in manageable amounts. A family member or trusted person can help record questions if you wish. Discuss how treatment fits your priorities.

Sources: patient

TREATMENT REFERENCE · 06

The names on your treatment plan

Useful terms for clinic and pharmacy.

CAPOX, XELOX and fluoropyrimidine

CAPOX and XELOX name the same capecitabine–oxaliplatin combination. Capecitabine is a fluoropyrimidine, a drug family that includes fluorouracil. It is not interchangeable dose-for-dose with another tablet or infusion.

Sources: nccp

Adherence and interruption

Adherence means following the agreed plan, including stopping for toxicity when instructed. Interrupting tablets and calling about a serious symptom is part of safe treatment, not a failure to comply.

Sources: nccp

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

A study of stage III colon cancer after surgery found better long-term outcomes with CAPOX than with the intravenous fluorouracil schedule studied. The duration decision and evidence for metastatic colorectal or postoperative gastric cancer need separate discussion.

Sources: no16968

PREPARE FOR THE CONVERSATION

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber's review. The patient page explains the checks and does not authorise treatment.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00321 v9. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00321 v9 · official regimen
- eviQ: patient information (supportive explanation; NCCP schedule takes priority)
- HSE: chemotherapy side effects and urgent advice

- NCI: nausea and vomiting
- NO16968: final survival analysis (2015)

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-04.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6719/321_v9_XELOX_.pdf

https://www.eviq.org.au/medical-oncology/colorectal/adjuvant-and-neoadjuvant/4019-adjuvant-capox-xelox-capecitabine-and-oxal/patient-information

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting

https://pubmed.ncbi.nlm.nih.gov/26324362/